

End-of-Life Care: May Comprehensiveness Include Physician-Assisted Suicide?

1. Summary of two position statements

Zlatko Anguelov

BACKGROUND

In the previous issue of *Currents* (vol. 3, # 4), Jeffrey Spiess, M.D., head of the Palliative Care Service at UI Hospitals and Clinics, emphasized the patient-driven, team-based approach to end-of-life care adopted by this service. He ended his essay by summarizing the interaction between the two main actors involved in decisions about care at the end of life—the patient and the physician—in the following terms:

"The field of Palliative Care is challenging and difficult. However, the key in the individual patient encounter is usually simple. Get to know your patients. Listen to them. Understand what they need. Try to imagine what you would want done if you were in their position. And then do it."

Now, what if what the patients would want done is help them die?

This possibility is at the center of an ongoing debate involving the medical profession, the political and legal institutions, and the society as a whole. The medical profession is at the center of the debate, because physicians are the only professionals expected—although not legally authorized, except in the state of Oregon—to provide expert assistance in terminating life on the patient's request.

While other groups or actors only debate the issue, physicians are asked to act by dismissing or granting their patient's request. The sanctity of the patient-physician relationship is put to its hardest test when the patient desperately asks—or had requested while still legally mindful—his or her physician to perform an act that is in blunt defiance of the physician's role to care and cure: *"I will never give a deadly drug to anybody if asked for it, nor will I make a suggestion to this effect,"* says the Hippocratic oath.

Taking Spiess's ethical stance as a starting point, *Currents* will publish a series of articles addressing the issues of physician-assisted suicide (PAS) as logically associ-

ated with end-of-life care. The articles will include position statements of physician organizations, data on physicians' attitudes about and experiences with end-of-life care and PAS, and opinions of bioethicists on the morality of PAS and voluntary euthanasia (VE).¹ You may send us your opinions as well; through *Currents* they will reach more than 7,000 fellow physicians.

POSITIONS

The American Academy of Hospice and Palliative Medicine (Academy) strongly supports respect for patient autonomy. It stresses, however, that there are limits to autonomy when it conflicts with beneficence toward the patient and society as well as the health care provider. Respect for autonomy includes the right to have interventions withdrawn or withheld upon the patient's request. The withdrawal/withholding of life-sustaining intervention is not considered VE by the Academy in current ethical and legal contexts.

Considering the reduction of severe patient suffering as its main obligation, the Academy believes that universal availability of comprehensive palliative care would greatly reduce the perceived need for PAS/E instead of rendering it an equal part of the available options. Physicians have an obligation to scrutinize carefully the source of requests for PAS in hopes of discovering ways to alleviate the patient's suffering. Special attention must be paid to emotional, psychological, and spiritual distress.

However, even the best palliative care cannot alleviate all suffering, and patients can still request help to die. This requires a very thorough evaluation and re-evaluation of the morality of PAS/E. Controversial areas include those regarding the likelihood of the "slippery slope" leading to involuntary E; patient-physician trust; and how society might be affected by a more open and supervised

¹Outside the U.S., VE is legally authorized only in the Netherlands and Belgium.

Euthanasia (E) = the intentional taking of a terminally ill patient's life through, for example, lethal injection. Some authors distinguish *voluntary E* (VE), where the patient requests to be killed; *nonvoluntary E*, where the patient is unable to request to be killed, and *involuntary E*, where the patient does not request to be killed.

Physician-assisted suicide (PAS) = providing medical help to enable a terminally ill patient to perform an act that is specifically intended to take his or her own life, for example, overdosing on pills as prescribed by the physician for that purpose. There is considerable difference of opinion as to whether PAS and VE are ethically different.

Withholding/withdrawing life-sustaining treatments = complying with a terminally ill patient's refusal of such treatment.

practice of PAS/E in contrast to the current covert, unmonitored, and illegal practice.

Should PAS/E become legalized in the U.S.—and it may well be—the Academy would promote the following precautions: a) these acts could only be entertained in the context of an adequate trial of comprehensive palliative care; b) no one should ever be pressured to act against his or her own conscience if asked to assist someone in dying; c) safeguards must minimize the potential for abuse, focusing in particular on the protection of more vulnerable groups and against undue family or external influences upon their patient preferences; d) financial pressures by third-party payors to terminate life would need to be forbidden; and e) involuntary PAS/E would never be acceptable.

In the practice of palliative care, the Academy believes, the appropriate response to the request for PAS/E is to increase care with the intent to relieve suffering, not to deliberately cause death. The Academy would promote an approach of noninterference and, above all, non-abandonment of the patient and family.

The American College of Physicians-American Society of Internal Medicine (ACP-ASIM) does not support the legalization of PAS. The arguments run as follows.

The medical profession's ethical traditions have always focused on healing and comfort. Assisting in suicide carries with it the risk of compromising the patient-physician relationship and the trust necessary to sustain it. Respect for patient autonomy and privacy is not so absolute to sufficiently justify assistance with suicide.

Moreover, why should medicine arrogate to itself the task of alleviating all human suffering? Our culture's goal of eliminating death is a false goal. The elimination of all human suffering is also a false goal that, if pursued, will ultimately lead to bad medical care. Rather, physicians need to use their skills to eliminate or alleviate the medical conditions that cause suffering at the end of life by palliating somatic symptoms, such as pain and nausea, and

psychological conditions, such as depression and anxiety. When patients continue to suffer despite the best efforts at palliation, physicians should vigorously pursue the alleviation of the symptoms, even at the risk of unintentionally hastening death. When the patient's suffering is interpersonal, existential, or spiritual, the task of the physician is to remain present, that is, to be compassionate.

Furthermore, the medical profession has the duty to safeguard the value of human life. If PAS is legally permitted yet restricted to the terminally ill adults with preserved decision-making capacity, it will inevitably raise legal concerns about arbitrary discrimination of vulnerable groups, such as the disabled, the poor, the elderly, etc. The practice then will probably broaden to include non-consenting and non-terminally ill individuals. And, since some patients cannot take pills or push buttons, they will appear to be victims of arbitrary discrimination unless the practice is broadened from PAS to E.

Finally, the power to prescribe PAS carries a profound potential for misuse and abuse. A formal role for physicians to assist patients with suicide in an era of health care cost-containment appears particularly troublesome. A broad right to PAS in a country with no general right to health care would be ironic. It also remains far from clear if any array of regulatory safeguards and guidelines can adequately function to oversee PAS practice and protect the vulnerable. Of special practical concern is the uncertainty in prognostication. More often than not, it is impossible to predict exactly how long a terminally ill patient has to live or to what extent cognitive capacity will be damaged or impaired by disease or injury.

Recognizing the weight of the arguments posed by the PAS legalization supporters in the public debate and that particular cases of PAS are medically and ethically challenging, ACP-ASIM nonetheless concludes that PAS should be legally prohibited. To the extent that this dilemma is partly due to the failings of medicine to adequately provide good care and comfort at the end of life, medicine should do better by strengthening end-of-life palliative care.